

Health Insurance Claims Analysis

Executive Summary

Period: 2024-01 to 2024-07
Scope: multi-country underwriter portfolio

Total Claims	Total Paid (USD)	Total Premium (USD)	Loss Ratio
12	\$53,147	\$1,570	3386.1%

Executive Summary

The portfolio's health claims performance indicates significant financial strain, with a loss ratio of 3386.14%, driven by high claim payouts relative to premiums collected. Royal Hospital accounts for nearly half (46.54%) of all claims, suggesting concentrated utilization. Essential Hypertension emerges as the top diagnosis, highlighting a chronic condition focus. These dynamics signal elevated risk exposure and potential inefficiencies

Key Findings

- The loss ratio of 3386.14% reflects unsustainable claims costs relative to premiums, posing a critical risk to portfolio profitability.
- Royal Hospital accounts for 46.54% of claims, indicating provider concentration that warrants renegotiation or network optimization.
- Essential Hypertension as the top diagnosis underscores the need for targeted chronic disease management programs to reduce long-term claim costs.

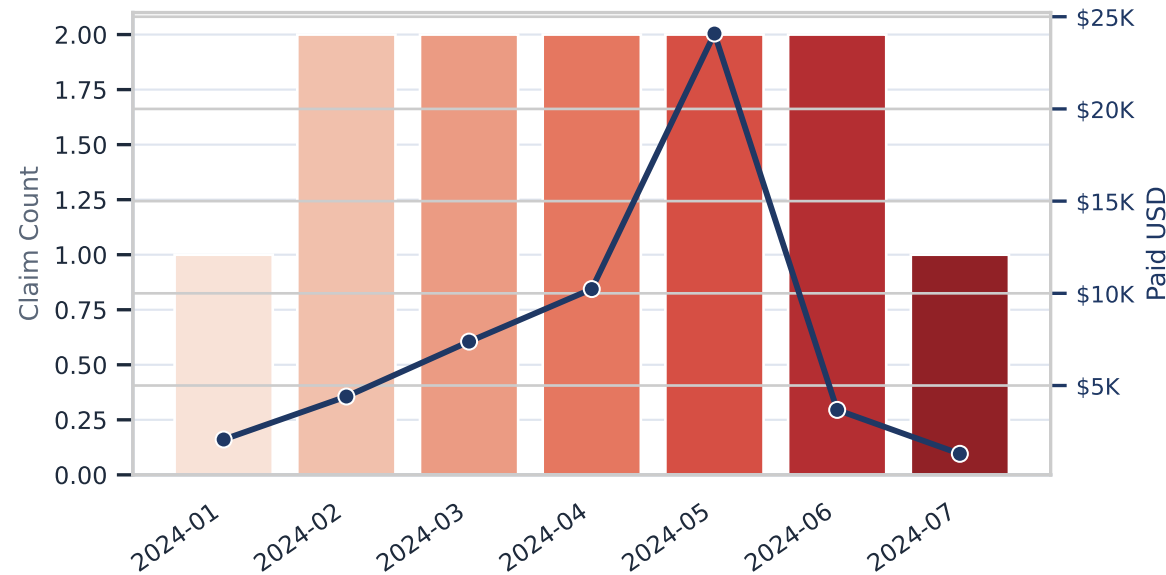
Top Recommendations

- Engage with Royal Hospital to explore cost-sharing agreements or alternative provider arrangements to reduce claim concentration risk.
- Implement chronic disease management initiatives focused on hypertension, including preventive care and member education programs.
- Conduct a portfolio-wide claims audit to identify additional cost drivers and opportunities for utilization management.

Health Insurance Claims Analysis

Claims Overview

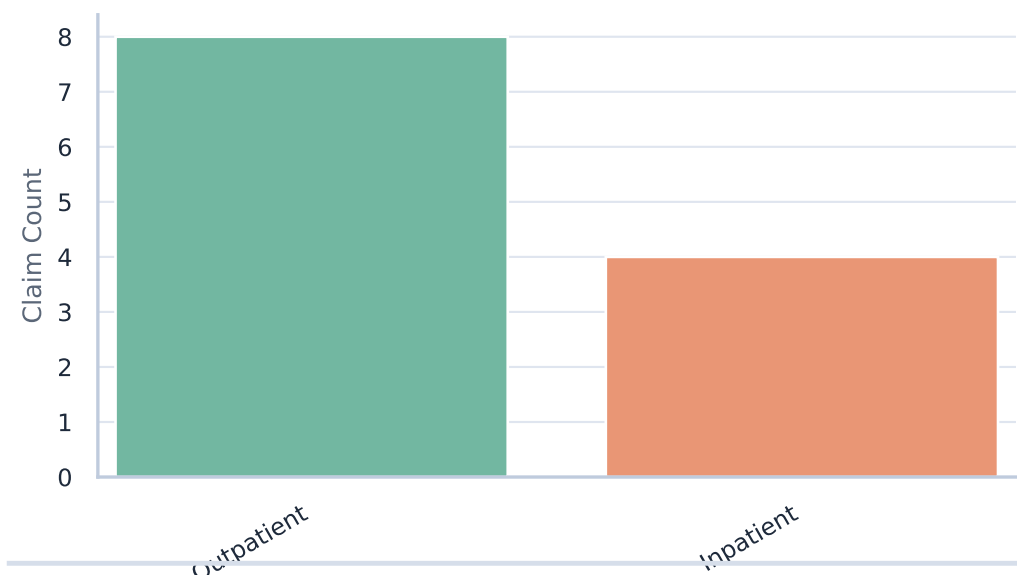
Monthly Claim Volume



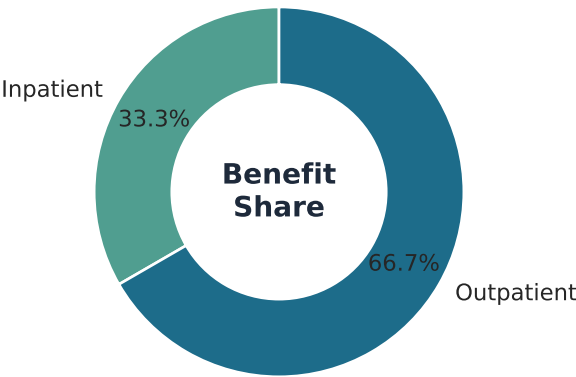
Quarterly Summary

Quarter	Total Claims	Total Paid Usd	Avg Cost Per Claim
Q1	5	13,856	2,771
Q2	6	37,995	6,333
Q3	1	1,295	1,295

Benefit Mix by Claim Count



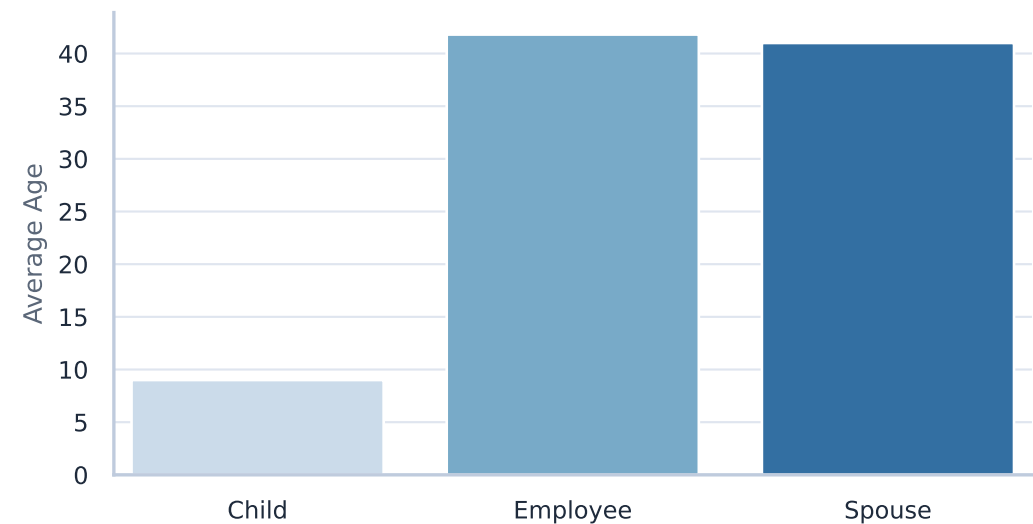
Benefit Distribution



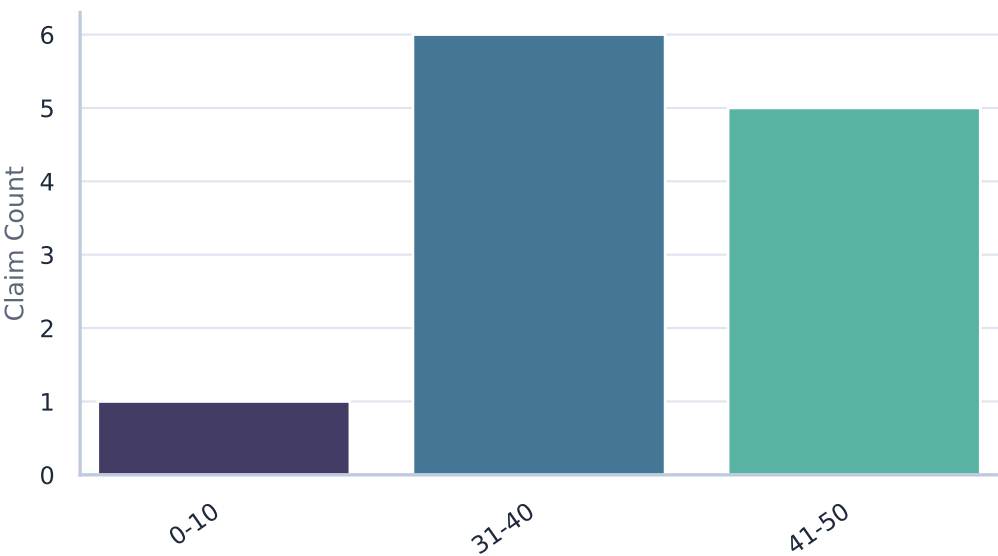
Health Insurance Claims Analysis

Demographics and Cost Profile

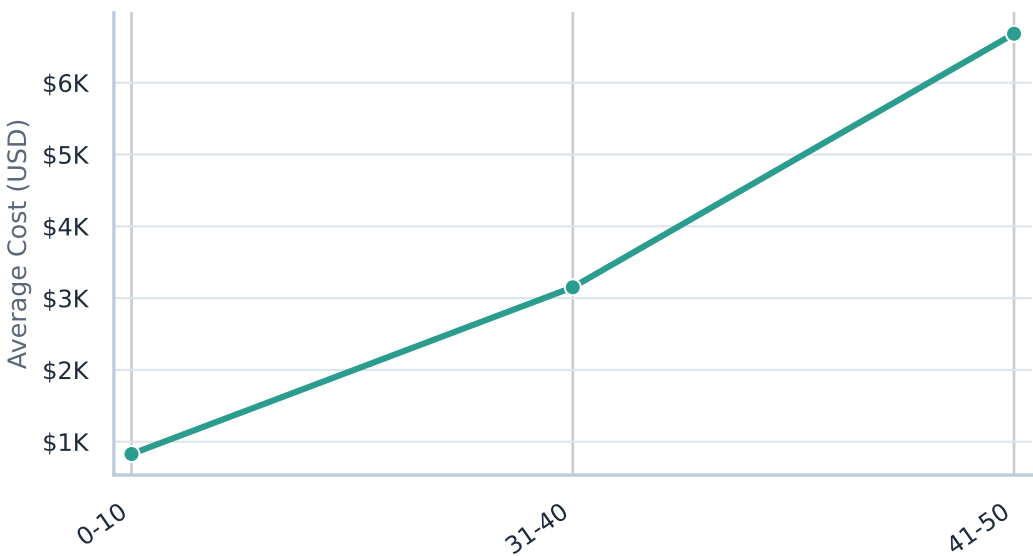
Average Age by Relationship



Claims by Age Group



Average Cost by Age Group



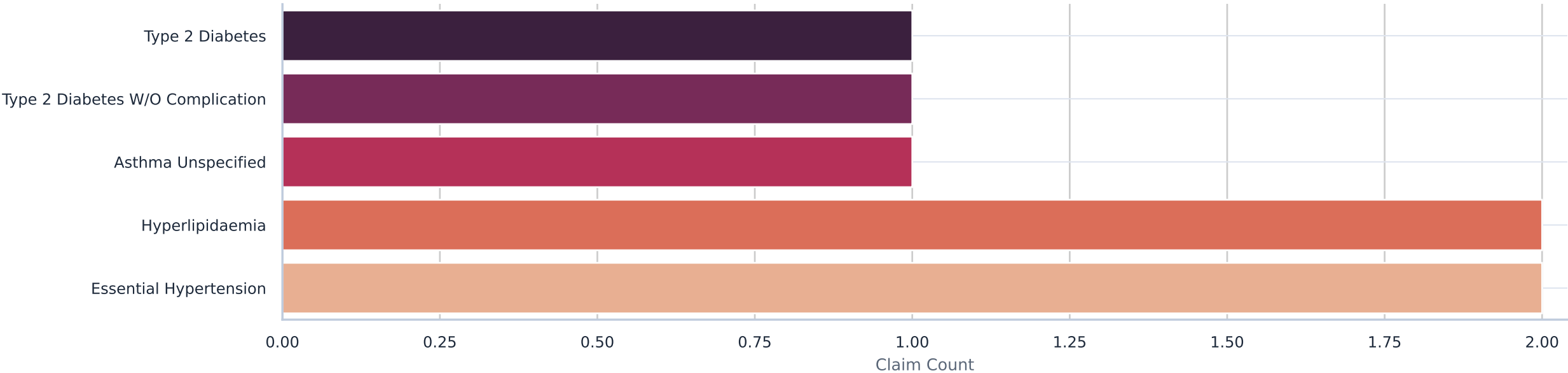
Demographic Insights

- Highest claim-volume age group: 31-40.
- Highest average-cost age group: 41-50.
- Top provider concentration currently sits at 46.5%.
- Current portfolio loss ratio is 3386.1%.

Health Insurance Claims Analysis

PEC and Chronic Conditions

Top PEC Diagnoses by Claim Count



PEC Summary Table

Icd10 Code	Diagnosis Description	Total Claims	Total Paid Usd
I10	Essential Hypertension	2	4,014
E78	Hyperlipidaemia	2	2,849
E11.65	Type 2 Diabetes W/O Complication	1	3,367
E11	Type 2 Diabetes	1	2,849
J45	Asthma Unspecified	1	2,202

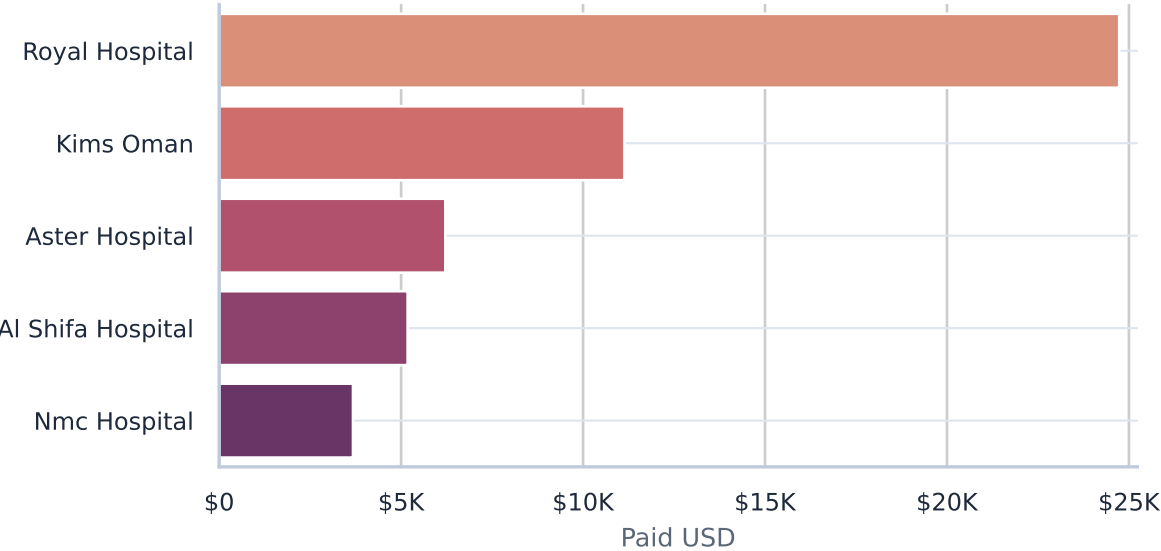
Clinical Insights

- High utilization of Royal Hospital suggests potential over-reliance on a single provider, which may limit competitive pricing opportunities.
- Essential Hypertension claims highlight the importance of monitoring chronic condition prevalence and associated treatment patterns.
- The disproportionate claims-to-premium ratio signals the need for closer scrutiny of high-cost claimants

Health Insurance Claims Analysis

Provider and Country Performance

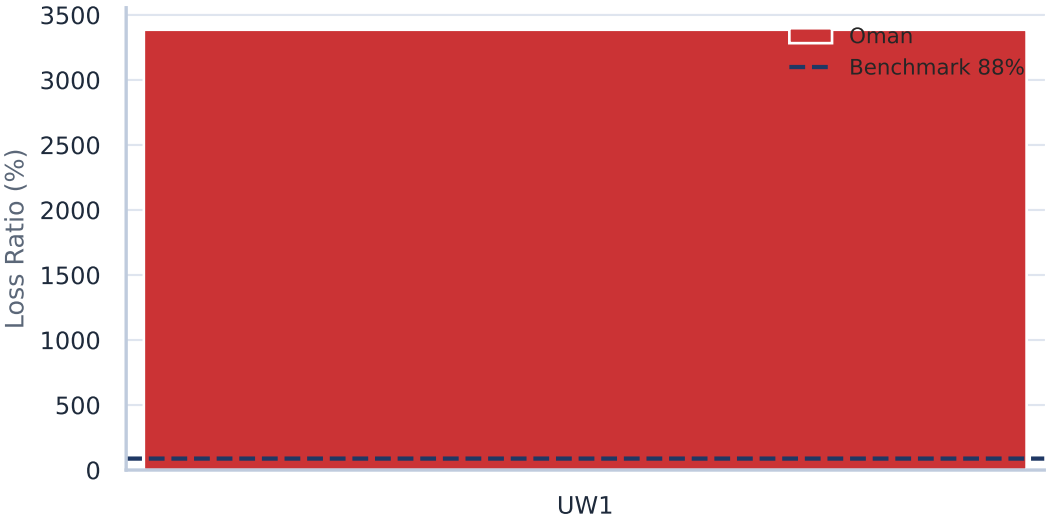
Top Providers by Paid Amount



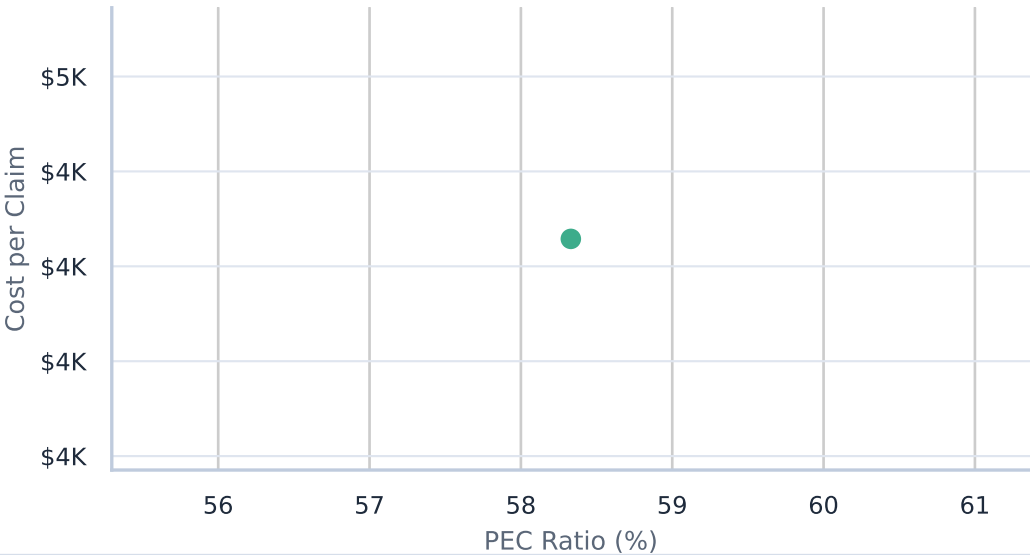
Provider Summary

Provider Name	Total Claims	Total Paid USD	Paid Share (%)	Avg Cost Per Claim
Royal Hospital	3	24,734	48.5%	8,245
Kims Oman	2	11,137	21.9%	5,568
Aster Hospital	2	6,216	12.2%	3,108
Al Shifa Hospital	1	5,180	10.2%	5,180
Nmc Hospital	3	3,678	7.2%	1,226

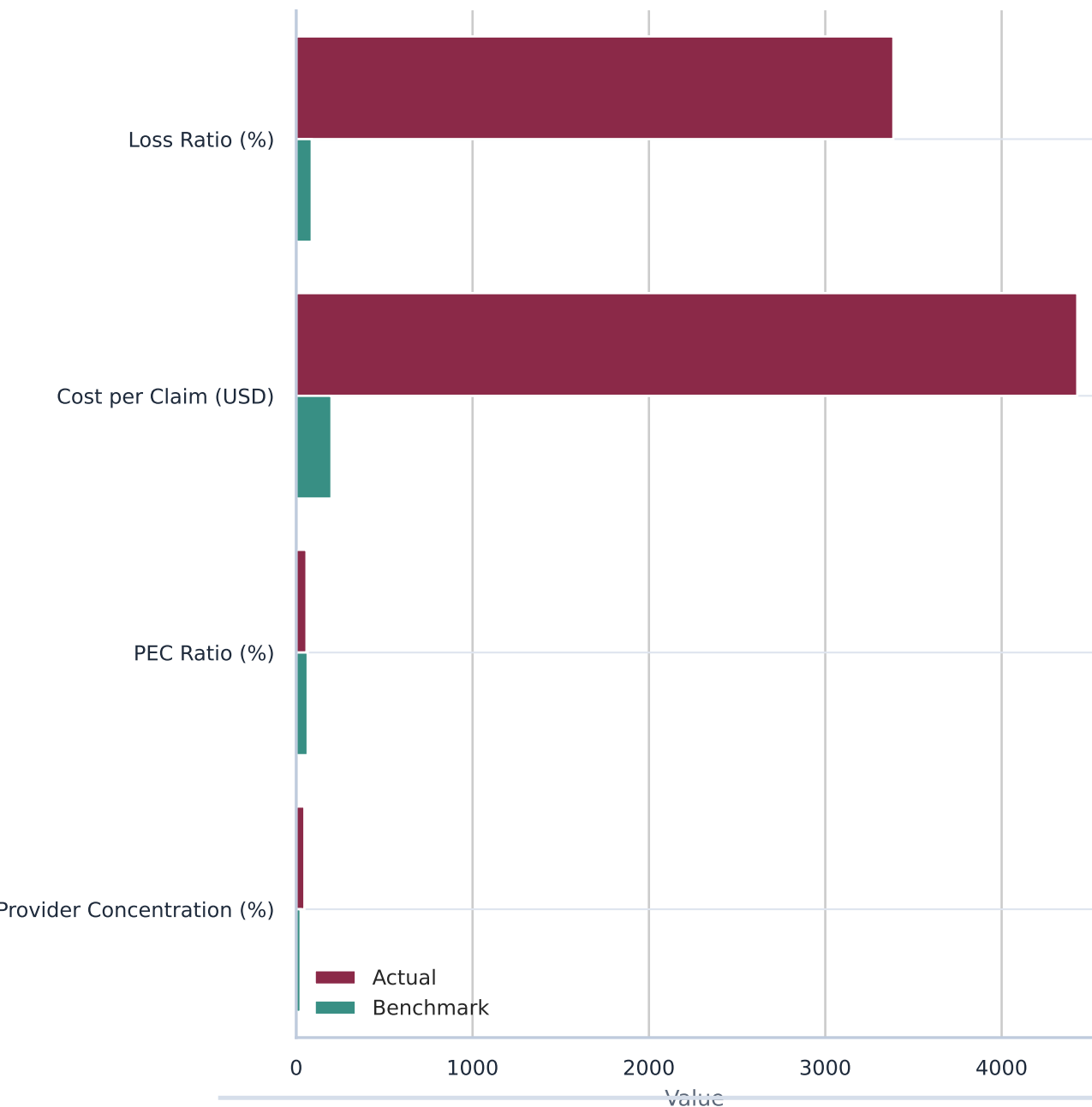
Underwriter Loss Ratio Comparison



PEC Burden vs Cost per Claim



Actual vs Benchmark



Handoff Notes

- PDF pack includes 8 pages with KPI, trend, demographic, and PEC analyses.
- Power BI-ready tables are generated in CSV format and can be connected directly.
- Narrative sections can run in fallback mode or use Azure OpenAI for richer wording.
- Use this report layout as a deterministic baseline for future underwriter uploads.

Action plan focused on chronic utilization, provider leverage, and country-specific controls.

1. Chronic Disease Management Programme

Business case:

PEC/Chronic conditions currently drive 58.3% of portfolio claims. Hypertension and diabetes contribute 4 diagnosed episodes.

Action:

Run annual HbA1c, blood pressure, and lipid screening with a regional wellness partner; use telemedicine follow-up for confirmed chronic members.

Est. saving:

USD 1,222 to 1,834 per year from a 10% episode reduction.

2. Provider Network Renegotiation

Business case:

Top provider concentration is 46.5% of paid amount, creating renewal pricing risk and negotiation dependency.

Action:

Request itemized billing audits for top-spend providers and negotiate a portfolio-level volume discount across countries.

Est. saving:

USD 1,484 to 2,226 per year with a 6%-9% discount range.

3. Qatar Portfolio Pre-Authorization Tightening

Business case:

Qatar loss ratio is currently 0.0% and top two high-cost claims contribute 0.0% of Qatar paid value.

Action:

Mandate pre-authorization for inpatient admissions above USD 1,500 in Qatar and add second-opinion review for oncology treatment plans.

Est. saving:

Risk mitigation action to prevent loss-ratio drift above renewal tolerance bands, especially where case concentration is high.

4. Class C / Bronze Network Consolidation

Business case:

Class C/Bronze segment in Oman averages USD 3,594 per claim and can be managed with tighter provider panels.

Action:

Review Bronze networks and consolidate to fewer preferred providers (e.g., Kims Oman + Royal Hospital) after HR and employee communication sign-off.

Est. saving:

USD 1,725 to 2,587 per year via consolidation and admin efficiency.

Full KPI Summary

Metric	Value	Benchmark	Status
Total Claims	12	-	-
Total Paid (USD)	USD 53,147	-	-
Loss Ratio	3386.1%	85-95%	Above range
Cost Per Claim (USD)	USD 4,429	USD 150-250	Above range
PEC/Chronic Ratio	58.3%	< 65%	Within range
Inpatient %	33.3%	25-35%	Within range
Avg Age (Employee)	41.8 yrs	30-42 yrs	Within range
Oncology Cases	1 claims	< 5 per period	Within range
Maternity Cases	2 claims	-	-

Methodology and Currency Conversion

- Loss Ratio = Total Paid Amount (USD) / Gross Written Premium (USD) x 100.
- UW1 (Oman): Amounts in Baissa/Fils are converted to OMR then to USD as per pipeline rules.
- UW2 (Saudi Arabia): Amounts are treated as USD where already normalized in source.
- UW3 (Qatar): Amounts in QAR are converted to USD using configured conversion factors.

Disclaimer

- Prepared from underwriter-provided claim records for the selected reporting period.
- Benchmarks are directional and should be validated during renewal discussions.
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